AVATAR DEEP DIVE RESEARCH REPORT - Jaw-Clenching / Night-Guard / Nighttime Spike

- generated_at: `2026-03-18 06:13 UTC`
- avatar_id: `jaw_clenching_nighttime_spike`
- config: `configs/avatar_states/jaw_clenching_nighttime_spike.json`
- workspace: `data/avatar_states/jaw_clenching_nighttime_spike`

Scope

This report is the scalable research wrapper around the active avatar workspace. It captures corpus scale, treated thread counts, state-cluster match counts, source coverage, and the long-form deep research body when attached.

Beachhead Definition

- label: `Jaw-Clenching / Night-Guard / Nighttime Spike`
- description: Beachhead tinnitus avatar modeled as a recurring state cluster: nighttime ringing, jaw tension or clenching, quiet-room amplification, sleep dread, night-guard confusion, and mechanism-hungry self-diagnosis.
- min_core_score: `5.5`
- min_core_states: `2`
- priority_min_core_score: `8.0`

Corpus Funnel

- normal threads in canonical corpus: `22456`
- orphan threads in canonical corpus: `1872`
- unique comments in canonical corpus: `252377`
- linked comments in normal threads: `246327`
- orphan comments preserved: `6050`
- quote candidates: `503620`
- broad avatar matches: `2797`
- priority avatar matches: `712`
- broad packs built: `254`
- priority packs built: `88`

Manifest Counts

- broad manifest rows: 2797
- priority manifest rows: 712

Source Coverage

- tinnitus: 2797 matched threads

Core State Counts

- quiet_room_amplification: 1892
- nighttime_spike_sleep_dread: 1884
- jaw_tension_clenching: 1760
- jaw_modulation_mechanical_link: 586
- night_guard_confusion: 413

Adjacent State Counts

- ent_dismissal: 1681
- stress_loop: 1322
- ear_fullness_pressure: 1043
- paper_normal_still_miserable: 628
- bite_change_dental_trigger: 617
- temporary_fix_burnout: 18

Top Broad Threads

- t3_1podzd6 | score 16.19 | This is what I wish someone had told me when I first got tinnitus about a year ago (My success story with tinnitus habituation)
- t3_1bhqi3q | score 16.19 | Hello! I'm an ENT AMA ask me anything
- t3_1pkmif9 | score 15.75 | How I cured my tinnitus
- t3_1mrj9zh | score 15.75 | Tinnitus after one year , it's so much better!
- t3_1mu9oiw | score 15.66 | I'm scared
- t3_1jt37q2 | score 15.66 | Accidentally fixed my long term tinnitus
- t3_1hb2wku | score 15.66 | I screwed up and now I have tinnitus
- t3_1etyw2j | score 15.66 | Found out my tinnitus was caused by TMJ and 5 years of torture are gone
- t3_1dsyr4j | score 15.66 | Tinnitus Resolved - success!!
- t3_1e0elry | score 15.31 | Where is everybody?
- t3_1cds6kg | score 15.31 | Lost
- t3_1k31cs5 | score 15.22 | Pitch and sounds change depending how I lay etc, does it happen to anybody else?
- t3_1apoz1m | score 15.22 | Tinnitus and Gastrointestinal Problems anyone?
- t3_1aitvqb | score 15.22 | Improvements in intermittent tinnitus, 22 months since onset

- t3_1fwi53m | score 14.88 | My tinnitus has basic vanished and I think I know the culprit.

Top Priority Threads

- t3_1podzd6 | score 16.19 | This is what I wish someone had told me when I first got tinnitus about a year ago (My success story with tinnitus habituation)
- t3_1bhqi3q | score 16.19 | Hello! I'm an ENT AMA ask me anything
- t3_1pkmif9 | score 15.75 | How I cured my tinnitus
- t3_1mrj9zh | score 15.75 | Tinnitus after one year , it's so much better!
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- t3_1fwi53m | score 14.88 | My tinnitus has basic vanished and I think I know the culprit.

Near-Miss Discovery Threads

- t3_1fxr9rp | score 6.4 | Spell out the sound of your tinnitus
- t3_1bxi0je | score 6.22 | Is my doctor bluffing?
- t3_1eh7bjm | score 5.88 | low frequency tinnitus. How common is it?
- t3_1bx342w | score 5.44 | It's getting worse, I don't think I will be able to do my job well today
- t3_1aytj0n | score 5.0 | Finally some relief!!
- t3_1bghlms | score 5.19 | I went to a 2nd ENT and saw an audiologist
- t3_1cyvwm5 | score 5.19 | Should i go to the doctor now?
- t3_1dwlb0k | score 5.19 | ENT appointment for my tinnitus tomorrow.
- t3_1f5yg98 | score 5.19 | HELP
- t3_1fqxmv2 | score 5.19 | My Experience With Prednisolone - As a Warning That It's Not a Cure All & to Be Careful With It.
- t3_1fx3r0w | score 5.19 | Tinnitus disappeared when applying oil
- t3_1hs5yui | score 5.19 | Did your tinnitus sneak up on you? Or did it appear suddenly?
- t3_1hvsq92 | score 5.19 | Which test is required for HEARING LOSS
- t3_1ji139g | score 5.19 | Only a few months in an already feeling hopeless (+ need advice)
- t3_1kpgx6u | score 5.19 | Today, I'm choosing a different kind of day.

Reproducibility

- seed corpus: `data/avatar_states/jaw_clenching_nighttime_spike/seed_corpus.csv`
- canonical threads: `data/threads/threads.jsonl`
- canonical orphan threads: `data/threads/orphan_threads.jsonl`
- quote candidates: `data/evidence/quote_candidates.jsonl`
- broad slice manifest: `data/avatar_states/jaw_clenching_nighttime_spike/thread_index.csv`
- priority slice manifest: `data/avatar_states/jaw_clenching_nighttime_spike/priority_thread_index.csv`

Deep Research Body

AVATAR DEEP DIVE RESEARCH

Avatar: Jaw-Clenching / Night-Guard / Nighttime Spike
Prepared for direct-response strategy
Date: 2026-03-18

Research Scope

This dossier is built from the local Reddit corpus and the active avatar-state workspace for the beachhead cluster:

- nighttime ringing
- jaw tension / clenching
- quiet-room amplification
- sleep dread / lying-down spike
- night-guard confusion
- normal-hearing-test-but-still-miserable

This is not a broad tinnitus persona. It is a state-cluster read for a direct-response beachhead.

Important:

- all major conclusions below are grounded in the local `r/tinnitus` corpus
- the user-supplied seed CSV adds `r/TMJ` and `r/bruxism` directionally, but those raw exports are not ingested locally yet
- claims in these threads are customer beliefs and experiences, not clinical proof

Quick Snapshot

Field	Finding
Beachhead avatar	Jaw-clenching / night-guard / nighttime-spike sufferer
Canonical corpus	22,456 normal threads, 1,872 orphan threads, 252,377 unique comments
Avatar broad slice	2,797 treated threads
Avatar priority slice	712 high-fit threads
Quote surface	503,620 canonical quote candidates; 168,907 inside the broad slice
Local subreddit coverage	`r/tinnitus` only in the current treated corpus
Date coverage in treated slice	2024-01-01 through 2026-03-17
Seed corpus overlap	29 manual seed rows supplied; 10 are present in the current local corpus and all 10 land in the current slice
Awareness read	late solution-aware, brushing product-aware in appliances / physio / Botox / posture work
Market read	mechanical-feeling problem; generic ear-health framing is a mismatch

Corpus Funnel

The treated funnel for this avatar currently looks like this:

Stage	Count
Canonical normal threads	22,456
Canonical orphan threads preserved	1,872
Unique comments	252,377
Broad avatar-state matches	2,797
Priority avatar-state matches	712
Priority share of broad slice	25.5%
Broad slice from orphan threads	39
Broad packs built	254
Priority packs built	88
Packs manually reviewed for this write-up	9

Most Repeated Patterns In The Treated Slice

Pattern	Count	Share of broad slice	Read
Quiet-room amplification	1,892	67.6%	Silence is not neutral; it is where the problem becomes emotionally loud
Nighttime spike / sleep dread	1,884	67.4%	Nights and lying down are a major battlefield
Jaw tension / clenching	1,760	62.9%	This cluster reads the problem through the jaw first, not just the ear
ENT dismissal	1,681	60.1%	“Normal” tests often push them deeper into self-diagnosis
Stress loop	1,322	47.3%	Stress is treated as fuel, not full explanation
Ear fullness / pressure	1,043	37.3%	The symptom set feels physical and positional
Paper-normal still miserable	628	22.5%	Normal hearing tests do not resolve the fear
Bite change / dental trigger	617	22.1%	Dental events and bite shifts are treated as real clues
Jaw modulation / mechanical link belief	586	21.0%	If movement changes the sound, they infer a fixable mechanism
Night-guard confusion	413	14.8%	Guards are framed as rescue, placebo, or backfire depending on the user

Top co-occurrences:

- quiet-room amplification + nighttime spike: 1,348 threads
- jaw tension / clenching + nighttime spike: 917 threads
- jaw tension / clenching + quiet-room amplification: 900 threads
- jaw modulation + jaw tension / clenching: 434 threads
- jaw tension / clenching + night-guard confusion: 404 threads

State-Cluster Model

This avatar is best understood as a loop, not a demographic.

1. Stress, posture, bite pressure, or overnight clenching load the jaw / neck system.
2. The ringing spikes most clearly in bed, on waking, when side-sleeping, while slouching, or when biting / yawning / turning the head.
3. Quiet rooms remove distraction and make the sound feel bigger than life.
4. Poor sleep and fear raise next-day tension.
5. The sufferer starts detective mode: ENTs, dentists, guards, splints, posture work, massage, Reddit, YouTube.
6. Mixed advice and mixed results create mistrust, expense fatigue, and mechanism hunger.

Avatar Explanation

This is the person who does not experience tinnitus as a random ear event. They experience it as a jaw-linked state problem. The sound gets framed through clenching, sleep position, pillow pressure, worn teeth, uneven bite, sore jaw, neck tension, ear fullness, and the strange fact that jaw or head movement can change the pitch or intensity. Because the hearing test often comes back normal, they do not feel reassured. They feel stranded between ENT, dentist, TMJ specialist, posture work, and self-research.

What they want is not generic "tinnitus support." They want a believable explanation for why the ring gets worse at night, in quiet rooms, or when the jaw tightens. They want quieter nights, less morning carryover, less dread, and a path that feels specific to their subtype.

Age

Observed age is too weak for a hard demographic claim. The local slice contains working-age behavioral signals, but not enough explicit self-reported ages to lock a reliable age band.

Read:

- use `null` in structured output
- plan copy for state-recognition, not age-recognition

Gender

Gender signal is also too weak for a reliable call. The local slice reads cross-gender and the state cluster is the better targeting layer than gender.

Read:

- use `null` in structured output
- keep launch framing gender-neutral

Pain Points

- Nights feel unsafe. The core pain is not just "I hear ringing." It is "the room went quiet, I lay down, and now it is louder." Bedtime becomes anticipatory fear.
- They often feel a full bundle rather than a single symptom: jaw tightness, clicking, ear fullness, pressure, soreness on waking, sound sensitivity, and positional spikes.
- "Normal" hearing results make the experience feel invalidated rather than resolved.
- Wrong-fix fear is intense. A guard, splint, dental change, or expensive TMJ program can feel like either the answer or the thing that makes it worse.

Representative language:

> "my T is unilateral, gets louder when I open my mouth wide"
> "Often quiet or barely noticeable in the morning, but much louder at night or when lying down"

> “my tinnitus went from manageable to louder than life”

Day-to-Day Struggles

- Constant body surveillance. They monitor jaw position, bite, clench pressure, pillow contact, side-sleeping, slouching, and whether the head turn or yawn changed the sound.
- Quiet work and screen-heavy posture worsen the day. Desk time, driving, or long stretches of stillness can turn into trigger environments.
- They live in experiment mode: store-bought guards, better guards, splints, PT, massage, posture corrections, muscle relaxants, nasal sprays, Botox, jaw exercises, neck work.
- The problem steals attention from both ends. Poor sleep makes them tense; the tension makes the ring feel louder; the louder ring makes sleep and concentration worse.

Representative language:

> “I constantly try to correct myself from slouching but it’s really hard.”
> “I have a long drive, I'm guessing my posture while I'm driving is affecting it too.”
> “this has been overwhelming and I’m trying to stay grounded.”

Victories

- The wins that matter most are usually partial, not absolute: less sharpness, less fullness, fewer waking spikes, lower perceived volume, more good days, or moments of forgetting about it.
- Physio / manual work / posture correction show up as the cleanest recurring small-win lane in the local slice.
- Some guards or splints help when they reduce clench intensity or jaw pressure; others fail. The point is not the object itself. It is whether it changes the underlying tension pattern.
- Hope spikes hard when the improvement feels subtype-specific. When relief comes through jaw / neck / posture work, sufferers feel they are finally “in the right lane.”

Representative language:

> “I could see 50% reduction in ringing sound.”
> “the ringing got less sharp and the fullness in my ears calmed down.”
> “I have also had three tinnitus free days”

Failures

- OTC or badly fitted guards can worsen clenching, create tooth pain, or spike the noise.
- Teeth protection and tinnitus relief are not treated as the same thing. Many users explicitly separate “protects my teeth” from “helps my tinnitus.”
- Botox, massage, orthotics, or PT may help tension, pain, or fullness without touching the ring itself.

- ENT loops often feel useless: normal audiograms, short consults, coping advice, no mechanism, no meaningful next step.
- Expensive TMJ packages trigger skepticism when the practitioner cannot explain why this appliance or protocol should work.

Representative language:

> “it only encouraged me to clench and caused pain.”
> “its primary purpose is to protect your teeth, nothing more.”
> “None of this has touched my tinnitus.”
> “they charge $600 for the consultation and $5,000 to $10,000 for treatment.”

Goals

- Figure out what is actually driving it: clenching, posture, bite, neck tension, stress, congestion, or some mixed case.
- Stop the bedtime / wake-up spike cycle.
- Get the sound low enough to stop dictating sleep, quiet time, work, and focus.
- Protect the teeth and jaw without worsening the ring.
- Escape detective mode and follow a believable plan instead of throwing money at guesses.

Representative language:

> “I wish everyone here will find their root cause too.”
> “Does this pattern sound like something that can improve or go away over time?”
> “anyone had any luck easing this shit with this kind of tinnitus? I need a lifeline”

Beliefs

- If jaw movement changes the sound, there must be a physical or mechanical component.
- Stress matters, but “stress” alone is not a satisfying explanation. They want to know what stress is doing to the jaw, muscles, bite, posture, or sleep.
- A normal hearing test does not mean nothing is wrong.
- Custom, context-specific help feels more legitimate than generic mass-market fixes, but trust is fragile because expensive “expert” help also fails often.
- Somatic or TMJ-flavored tinnitus feels more hopeful than random tinnitus, but not easy.

Representative language:

> “because the tinnitus's sound changes due to pressure on the jaw, the opposite is in theory possible to reduce or stop it.”
> “Multiple hearing tests — all normal”
> “the difference between a custom appliance/guard and one purchased from the store ... is that the custom appliance creates a level surface”

Desires

- I want to stop waking up to a loud ring and sore jaw, so I can start the day calm, so I can stop feeling ambushed by my own body.
- I want to know whether this is really jaw / clench / posture linked, so I can stop gambling on random fixes, so I can finally follow a plan that makes sense.
- I want the noise low enough to disappear into the background in bed and in quiet rooms, so I can sleep and think normally, so I can feel mentally free again.
- I want relief that does not mess up my bite or create a new problem, so I can trust my routine, so I can stop fearing the next “solution.”

Objections

- “How can this help if my problem feels mechanical?” This is the biggest supplement challenge. The buyer thinks in terms of clench pressure, bite alignment, posture, and nerve irritation.
- “What if it makes it worse?” This objection is real because many have already had a guard, splint, exercise, or dental move backfire.
- “I’ve already paid for guesses.” They do not want another vague package or explanation-light offer.
- “I’m not even sure mine is purely jaw-related.” Mixed-cause uncertainty makes them slow to commit.
- “If it only masks the sound and doesn’t calm the loop, I won’t trust it.”

Representative language:

> “they are not giving a clear picture and just gave a random package deal”
> “I was fitted with a mouthguard ... and now I have tinnitus that hasn't gone away”
> “I don’t know what to believe”

Level of Awareness

Best read: late solution-aware.

Why:

- they already know the main solution buckets: ENT, hearing tests, sound masking, guards, splints, PT, massage, Botox, posture work, dental work, muscle relaxants
- they do not need to be told the problem exists
- they need help sorting mechanisms and deciding which lane actually matches their subtype
- some are brushing product-aware around appliances and specialists, but the market does not show one trusted “default” product answer

Emotions & Identity

- Control: they want to feel they can influence the sound instead of waiting to be hit by it in bed.

- Validation: a normal audiogram with abnormal suffering makes them feel unseen, dismissed, or half-gaslit.
- Competence: they want to feel smart for finally decoding their subtype after conflicting advice.
- Safety: what they miss most is not only silence; it is the feeling that nighttime is not a threat.
- Normalcy: they want a boring night, a boring morning, a boring quiet room. That is the dream.

Representative language:

> “constant sense of dread”
> “I was having panic attacks multiple times a day and suicidal.”
> “I am just afraid, and I don't know what to believe”

Real Customer Language

Phrase	Why it matters
“louder than life”	This is not a mild-annoyance frame. It describes a panic-level jump.
“constant sense of dread”	The ring is tied to anticipation and fear, especially at night.
“I need a lifeline”	They are not browsing casually; they are asking for rescue.
“perfect hearing test”	This phrase captures the gaslighting problem in five words.
“ear was clogged” / “fullness”	The problem feels physical, not imaginary.
“someone screaming in my ears for 48 hours”	This is how they describe spike severity when it becomes unbearable.
“protect your teeth, nothing more”	They already distinguish between partial protection and true subtype relief.

Visual Cues

- One cheek pressed into the pillow and the sound instantly spiking.
- A person waking up and touching the jaw before even getting out of bed.
- A night guard or splint on the nightstand next to white noise that still did not solve the problem.
- A “perfect” hearing test paper next to someone still hearing a scream in the ear.
- Ear fullness, jaw clicking, and ringing showing up together as one morning pattern.
- The sound described as a smoke alarm chirp, crickets, or a long “eeeeeeeeeeee.”

Trigger Moments

- They wake up with a jaw-sore morning spike.
- They notice the ring changes when they clench, yawn, open wide, move the head, or sleep on one side.

- A dentist points out grinding, bite shift, chipped molars, or jaw wear.
- An ENT says hearing is normal and offers little beyond coping.
- A new mouth guard, splint, aligner, wisdom-tooth event, or dental adjustment changes the sound.
- A run of bad sleep, stress, congestion, or prolonged desk posture makes the loop flare.

Emergent Adjacent Clusters

The system also surfaced adjacent sub-clusters worth tracking as expansion lanes:

1. Paper-normal still miserable The tests say "fine," but the subjective suffering is severe.
2. Bite-change / dental-trigger tinnitus Wisdom teeth, Invisalign, fillings, orthotics, or mouth guards become the start of the story.
3. Posture / neck chain Forward-head posture, desk work, long drives, or cervical tension look causal to the user.
4. Ear-fullness / pressure subtype The ringing story is inseparable from clogged, pressurized, or physically "wrong" ear sensation.

These are not separate markets yet. They are adjacent entry points and likely future sub-avatars.

Early Creative Takeaways

- Lead with the pattern, not the diagnosis. "Ring gets louder when your jaw clenches at night" will out-pull generic tinnitus language.
- Do not promise silence. The believable promise is quieter nights, lower morning carryover, less jaw-tension spillover, and more manageability.
- Never talk like a generic ear-health brand. This buyer thinks in jaw / tension / posture / bite / sleep loops.
- A supplement angle only works if it bridges the gap between "mechanical-feeling problem" and "systemic state support."
- The believable lane is support for the night-clench / stress-reactivity / recovery loop, not "we fix your ears."
- Acknowledge guard confusion directly. This audience already knows guards can help, do nothing, or backfire.
- Show the whole cluster in copy: sore jaw, fullness, pillow pressure, quiet-room spike, normal test, no plan.

Appendix A: Thread Snapshots

ENT told me to do nothing about my TMJ Tinnitus

What happened:

- user has unilateral tinnitus, jaw pain, clicking, head-turn modulation, normal hearing tests
- dentist sees grind pattern and bite shift

- ENT acknowledges TMJ, then offers no meaningful plan
- replies split between “see a specialist,” “OTC guards can backfire,” and “custom guard wasted my money”

What it reveals:

- this buyer hates dead-end reassurance
- the guard conversation is already nuanced
- “normal hearing” does not remove urgency

If your tinnitus gets worse when sitting down or laying down, it’s probably because it’s from TMJ

What happened:

- original poster maps spikes to sitting, looking down, pillow pressure, and sleep clenching
- commenters immediately self-identify
- the conversation becomes practical: muscle relaxants, splints, jaw soreness on waking, temporary spike during exercises

What it reveals:

- positional triggers are identity triggers
- bedtime mechanics are not fringe detail; they are central to the avatar

Day 8 tinnitus: quiet in morning, loud at night

What happened:

- new sufferer with normal tests, TMJ history, congestion, anxiety, and louder nights
- asks whether the pattern sounds temporary and how to survive the early days mentally
- replies split between reassurance, medication, habituation, and research links

What it reveals:

- the first conversion moment is often not “buy relief,” but “tell me I’m not doomed”
- early-state sufferers are desperate for pattern confirmation

Bruxism (or TMJ?) caused tinnitus

What happened:

- poster wakes up with intense tinnitus, no loud sound history, dentist discovers long-term grinding
- core question: can grinding be the only symptom and will a mouth guard help?
- replies explain the split: some get jaw relief, some get no tinnitus change, some improve when clench pressure falls

What it reveals:

- hidden bruxism is a real entry point

- lack of jaw pain does not disqualify the cluster

Does your tinnitus get significantly louder when you clench your jaw?

What happened:

- poll thread becomes a live map of modulation
- users report louder pitch with clenching, pressure, yawning, head turns, push-ups, and morning without a guard
- comments immediately shift into mechanism theories, hope, and self-experiment ideas

What it reveals:

- modulation is proof language for this avatar
- if the sound changes with movement, they assume there is leverage somewhere

TMJ Tinnitus? Will it go away?

What happened:

- bedtime onset, head-turn trigger, left-ear ringing, distress, normal ENT, TMJ consult wait time
- user weighs insurance-covered specialist versus same-day private clinic charging thousands
- replies add more mixed-cause stories: Invisalign, wrong bite, slouching, side-sleeping, mouth guard tradeoffs

What it reveals:

- urgency + uncertainty + price pain is a major decision state
- this buyer is highly vulnerable to expensive, explanation-light offers

Has anyone found relief from tinnitus stemming from TMJ or jaw related issues?

What happened:

- user already wears a night guard and is researching Botox
- replies describe oral splints, weekly adjustments, PT, massage, posture correction, and partial wins
- key theme: some relief is real, but outcomes are mixed and expensive

What it reveals:

- they are open to solutions
- they are not open to magic
- “partial but believable” beats “dramatic but vague”

Somatic tinnitus reduced by 50%

What happened:

- poster reports 50% reduction after jaw / neck / shoulder physio work
- replies ask whether traction, laser, cupping, or manual release did the job
- the thread becomes a negotiation between hope and uncertainty

What it reveals:

- even when people improve, attribution stays messy
- this market rewards believable progress, not certainty theater

Source Coverage

- treated broad slice: 2,797 threads
- treated priority slice: 712 threads
- manual pack review for this write-up: 9 pack files
- subreddit coverage in local treated corpus: `r/tinnitus`
- date coverage in local treated slice: 2024-01-01 through 2026-03-17
- seed corpus supplied by user: 29 rows across `r/tinnitus`, `r/TMJ`, `r/bruxism`
- seed rows present in current local corpus: 10
- seed rows present in current local avatar slice: 10

Gaps And Unknowns

- The local treated corpus is still `r/tinnitus` only. `r/TMJ` and `r/bruxism` raw exports should be added next.
- Demographics are too weak to lock age or gender confidently.
- The local export appears heavily 2024-2026 weighted, so older Reddit history is underrepresented.
- Some strong relevant rows survive only as orphan threads because the submission body is missing from the export.
- Supplement-specific objections are directionally inferable, but direct supplement discussion is sparse. The strongest available proxy is the avatar's mechanical-fix mindset and fatigue with vague wellness language.

Next Research Questions

1. When `r/TMJ` and `r/bruxism` exports land, does night-guard confusion rise enough to split into separate device-positive and device-burned sub-clusters?
2. Are there stronger female-skewed sleep-disruption patterns inside the broader cluster once non-`r/tinnitus` sources are added?
3. Which language shows up most often right before money is spent: "specialist," "splint," "custom," "orthotic," "Invisalign," "Botox," or "physio"?
4. Does the mechanical-belief cluster break into two lanes: jaw-first and neck/posture-first?
5. Which promise territory is most believable for cold Meta traffic: quieter nights, less morning spike, less jaw-tension carryover, calmer stress-clench loop, or all of the above framed as one state-reset story?